

TARUN KHANNA: The problems don't stop there.
There are many institutional voids that complicate the appropriate delivery
of health care.

I'm going to stick with the imagery of my needing
to find a doctor to treat me for some condition
that I've just found that I had.

Let's say that I even found the doctor.

Once I go to see her, how do I know that the facilities within which
she's

operating are basically kept clean?

How do I know that the sterilization that
should be there in the environment in which I'm seeing the doctor
is appropriate-- something as basic as that?

Of course, you'll find people running around.

Labor is cheap in many developing countries.

You'll find people running around cleaning things.

But the most basic things-- maybe their hands won't be sterilized
themselves.

I don't have any ability to ensure that the facility in which I'm being
treated

is adequately maintained.

And there's no reassurance that that's the case.

A version of that would be how do I know that the materials that
are being used, the equipment that's being used,
the consumables that are being used, the medicines, the drugs are
appropriate?

How do I know that they're not pirated?

How do I know that they haven't been adulterated?

You can easily imagine that a variety of health care providers could make a fair amount of money by adulterating the medicines or by delivering fake medicines.

And we know from global intellectual property estimates, policing estimates,

that the problem of piracy and drugs is rampant.

Now, I would hasten to say that I'm not implying for a second that doctors are unethical or nurses or unethical in developing countries--

far from it.

I think they are heroes of our time.

But it is the case that as a patient, I am going to be skeptical and I am going to come in with a view to saying, how do I know that what you are using to treat me is appropriate, that the facilities in which you've treated me, particularly if it's an operating room type of setting for a minor or even, God forbid, a major surgery, how do I know that it's been maintained in the right way?

Now, again, in my backyard here in Boston, I would have complete reassurance.

The penalties for somebody adulterating drugs or misrepresenting the chemical composition of something that they're about to put into my body are so severe and the adjudication mechanisms available to me if I have a dispute or something, god forbid, goes wrong with me are so well established, that I think even somebody who was inclined to behave ethically would think twice, thrice, 10 times over before he or she did something like that.

So there are mechanisms here that protect the patient, reassure the patient, and therefore make it much more likely that their health transaction will occur.

Those mechanisms are missing in most developing countries or are inadequately developed.

Even if there are rules on the books, they're not adequately enforced and maintained and updated and so on. There isn't a machinery that exists to do all that.

And that, again, is an institutional void.

Remember, I spoke about information being missing that prevents the doctors and patients from finding each other?

You also have this basic problem of ensuring the sanctity of that health transaction.

That when that transaction occurs, when I go and pay for services or receive services, receive medication, I need to be reassured that that transaction is happening the way it's supposed to be happening.

I need to be reassured that the drugs that I'm taking are doing what they're represented to be doing.

And I also need to be sure that if something goes wrong, that I have a redress mechanism, an adjudication mechanism, someone

that I can go to, a regulator, an ombudsman, a patient's advocate, something like that.

These are all things that exist in my backyard here in Boston. But they don't exist in other parts of the world.

